

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT INFORMATION****

Name: Patient 17

Age: 77 years old | Sex: Male

MRN: 000-TEST-17

Date of Admission: 06/10/2026

Date of Discharge: 06/17/2026

Attending Physician: Dr. M. Richardson, Internal Medicine

****PRIMARY DIAGNOSES****

- Chronic Obstructive Pulmonary Disease (COPD) exacerbation, moderate severity
- Hypoxemic respiratory insufficiency
- Anemia of chronic disease
- Chronic pain syndrome, lumbar region

****RELEVANT FINDINGS****

Pre-bronchodilator spirometry: FEV1/FVC ratio 38% (GOLD Stage III – Severe airflow limitation). Admission SpO2 88% on room air; discharged on home O2 at 2L/min continuous with pulse-ox target $\geq 88\%$. CXR showed hyperinflation, no acute infiltrate.

****HOSPITAL COURSE****

Patient 17 presented with acute dyspnea on exertion (DOE), increased sputum production, and SpO2 85% RA. Initial management included IV methylprednisolone 125 mg q.6h $\times$ 3 days, then transitioned to oral prednisone taper (see below). Nebulized albuterol/ipratropium q.4h $\times$ 48 hours, then transitioned to scheduled inhalers. Supplemental O2 titrated to SpO2 goal; chest imaging and sputum culture obtained (negative). Patient tolerated diet

advancement. Ambulated with supervision. Discharged in stable condition with clear safety precautions.

****DISCHARGE MEDICATIONS****

| Medication | Dose | Frequency | Duration |

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| Tiotropium (LAMA) | 18 mcg | Once daily (AM) | Ongoing |

| Formoterol/Budesonide (LABA/ICS) | 160/4.5 mcg | Two puffs BID | Ongoing |

| Albuterol rescue inhaler | 2 puffs | PRN q.4-6h SOB | Ongoing |

| Prednisone taper | 40 mg Day 1–3, 30 mg Day 4–6, 20 mg Day 7–9, 10 mg Day 10–12, 5 mg Day 13–14 | Once daily (morning with food) | 14 days |

| Ferrous sulfate | 325 mg | Once daily (q.d.) | Ongoing |

| Acetaminophen | 650 mg | q.6h PRN pain | Ongoing |

| Docusate | 100 mg | BID | PRN constipation (especially with opioids) |

****ACTIVITY & MOBILITY****

Ambulate as tolerated with rest breaks. Avoid strenuous exertion, climbing stairs without handrails. Pulmonary rehabilitation strongly encouraged.

****DIETARY RECOMMENDATIONS****

No specific restrictions. Encourage adequate hydration (goal 1.5–2L/day) to help mobilize secretions. Small, frequent meals if dyspnea interferes with intake.

****FOLLOW-UP APPOINTMENTS****

1. ****Pulmonology clinic**** – in 2 weeks (with Dr. Helena Torres) for repeat spirometry, inhaler technique assessment, consideration of long-term O2 therapy optimization
2. ****Primary Care**** – in 1 week for prednisone taper monitoring and anemia reassessment
3. ****Home Health Nursing**** – Visit within 48 hours for O2 equipment check and medication education

****RED FLAG SYMPTOMS — SEEK EMERGENCY CARE IF:****